



UroPharma

Verified Investment Assessment

Preliminary Technology and Investment Assessment — Verified
Private and Confidential

NOT INVESTMENT ADVICE | HUMAN GATE REQUIRED

HEADLINE VERDICT

The company and the device are **real**, but the investment story is **materially overstated**, and the core product is **not regulatory-cleared**. Revised status: **NOT investment-ready; proceed to formal due diligence only**. On equity fundamentals this is a speculative, capital-hungry, pre-approval medtech in a crowded field with no comparative clinical data. The rational case for a small angel rests largely on EIS downside relief plus a low entry price and a credible near-term CE-mark milestone that the £500k actually funds.

Relationship flag: Andrew Sharkey is a director of UroPharma (Corporate Affairs Officer) and John's contact. That is an interest to declare, and also a candid diligence channel to use.

VERIFIED (this is not a phantom)

- **UROPHARMA LIMITED**, company number 08083943, incorporated 25 May 2012, Norwich. A real 13-year entity, not a shell. Insist paperwork names this entity (a separate "UROPHARMA, INC." and international namesakes exist).
- Device **dtb-instilla** (direct-to-bladder instiller) is real. **Scott Glickman** real, director since 2012 (professorship title unverified; public sources list Howard Shapland as CEO — clarify).
- **Andrew Sharkey** real director; **Laurence Stewart** real urologist (advisor/testimonial). The cited BJCP paper is real — but a commentary, not a clinical trial.

OVERSTATED OR COLLAPSED UNDER SCRUTINY

1. **US\$18bn market not credible**. Sub-figures are whole drug markets (UTI ~£8bn, overactive-bladder ~£4bn, bladder-cancer ~£2bn); "pain & inflammation £4bn" matches no real market of that size. UroPharma sells the delivery device, not the drug. Real addressable niche is low single-digit billions, small share achievable.

2. **“First Licensed” contradicts “awaiting CE Mark.”** Both cannot be true. The device is not CE-marked = not lawfully sellable in the EU; the UK route leans on the same clearance. Binary gating risk.
3. **Novelty is narrow; the field is crowded and validated.** Intravesical instillation is standard of care. Cleared competitors exist: UroGen Jelmyto (FDA 2020), J&J/TARIS TAR-200/Inlexzo (FDA approved Sept 2025), Combat Medical HIVEC. A dual-channel drain-then-instil catheter is incremental.
4. **The £3.5m raised and grant funding could not be independently corroborated.** No trace found; the one public reference is a US\$25m ambition. “Multinational patent protection” unconfirmed, with only 10% of the raise to patent development.
5. **No comparative clinical outcome data.** The cited commentary supports the concept, not this device's superiority.

THE REGULATORY GATE (the binary risk)

The device is not CE-marked. Under EU MDR, a drug-delivery catheter may be a drug-device combination requiring a Notified Body and possibly a medicine authorisation — months to years, and can fail. Decisive diligence question: **what exactly is instilled, and who supplies it?** Device-only (light path) versus integral drug-device (heavy path, drug-approval risk). This determines everything downstream.

THE INVESTMENT READ

- Pattern: £3.5m consumed, still pre-clearance and pre-revenue, now a small £500k bridge before a £3–4m Series A. Bridge behaviour; heavy future dilution for a fresh angel.
- Valuation sense-check: defensible pre-money ~£2–4m; above ~£4–5m is overpriced given no clearance, revenue or data.
- **EIS is the real driver:** ~30% income tax relief plus loss relief roughly halves the downside for a top-rate taxpayer — but conditional on the company qualifying, advance assurance in hand, and the investor's tax position. EIS reduces loss; it does not create return.
- Honest probability (judgement): majority chance of substantial or total loss; small chance of a strong outcome.

THE EIGHT DUE-DILIGENCE ASKS (via Andrew)

1. Is it CE-marked, yes/no? If “licensed,” by whom and evidenced how?
2. What is instilled and who supplies it?
3. Patent numbers, register extracts, freedom-to-operate opinion.
4. Proof of the £3.5m and the grant.
5. HMRC EIS advance assurance letter.
6. Cap table, burn, runway, prior valuations, and the milestone the £500k reaches.
7. Who is actually CEO, and confirm the professorship.
8. Any comparative clinical data.

Decision Status: NOT investment-ready. Proceed to formal due diligence only. If DD shows the £500k does not reach a fundable inflection, if the price is above ~£4–5m pre-money, or if EIS advance assurance is not in hand, the answer is no. Human gate and Shane's alignment required before any cheque. Meridian informs the decision; it does not sign it.